

February 23, 2026

Re: Request for Information: HHS Health Sector AI RFI

To: Department of Health and Human Services, Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology,

On behalf of Abridge, we appreciate the opportunity to comment on the HHS Office of the Deputy Secretary in collaboration with ASTP/ONC Request for Information (RFI) on what HHS can do to accelerate the adoption and use of AI as part of clinical care. Abridge is grateful for the chance to contribute to this vital conversation regarding what HHS can do in this area.

Founded in 2018, Abridge is the leading enterprise-grade AI platform for clinical conversations. Powered by a purpose-built system of intelligence for healthcare, the company is building real-time connections between patients, providers, and payers, starting from clinically useful and compliant documentation, and extending through prior authorization, risk adjustment, and clinical decision support. This year, Abridge is projected to support more than 80 million patient-clinician conversations across 250 of the largest and most complex health systems in the U.S., including the Department of Veteran Affairs. With deep EHR integration, support for 28+ languages, and 50+ specialties, Abridge is used across the entire care journey, including outpatient, emergency department, inpatient, and nursing workflows.

Abridge is setting the industry standard for the trusted deployment of AI across health systems. Features like Linked Evidence map AI-generated clinical documentation to source data, helping clinicians quickly trust and verify the output.

Abridge was awarded Best in KLAS for Ambient AI in 2025 and 2026, in addition to other accolades, including TIME Best Inventions of 2024 and 2025, CNBC Disruptor 2025, and Fast Company's Most Innovative Companies 2025.

With deep integration into the operational and clinical workflows of some of the largest and most complex healthcare systems in the US, Abridge has a unique perspective. Our partner experience team works closely with these institutions, giving us direct visibility into the daily friction points for both patients and caregivers. Patients and caregivers are looking to AI to address fundamental healthcare challenges, primarily through improved diagnostic accuracy, personalized treatment plans, and reduced administrative delays. They specifically hope AI will facilitate early disease detection such as identifying anomalies in medical imaging or predicting cardiac events and help manage complex, rare conditions by surfacing evidence-based insights that a human clinician might miss. There is also a strong desire for AI to lower the administrative burden on the patient-provider relationship; by automating clinical documentation, AI can return valuable time to clinicians, allowing for more focused, face-to-face patient engagement rather than screen-centered care.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

To incentivize and effectively use AI in clinical care, HHS should prioritize transitioning from fee-for-service models to outcomes-oriented payment frameworks and clarifying regulatory definitions that enable healthcare to fully capture the gains of recent breakthroughs in agentic AI. By aligning financial incentives with clinical results and modernizing standards for autonomous systems, HHS can reduce administrative burdens and foster a more innovative health technology ecosystem.

Priority Regulatory and Programmatic Changes

HHS has already taken important steps to advance interoperability, promote value-based care, and modernize data exchange standards. We support these efforts and believe the next phase of policy development should focus on ensuring that existing frameworks fully enable responsible, enterprise-grade AI deployment in clinical care.

Based on our experience, we believe that the following structural and policy modernizations would do the most to accelerate adoption and unlock new opportunities:

- **Codifying "Autonomous Agents" in Information Blocking Rules (45 CFR § 171.102):**
HHS should explicitly include automated means and autonomous AI systems acting as

clinician "agents" within the definitions of "access" and "use". This clarity would eliminate the technical or contractual ambiguities that can work to block AI-enabled workflows. Building upon the existing Information Blocking framework, clarifying this interpretation would reduce technical or contractual ambiguities that can unintentionally impede AI-enabled workflows, while remaining consistent with the statute's intent to prevent inappropriate interference with information exchange.

- **Narrowing the "Infeasibility Exception" (45 CFR § 171.204):** Current rules allow for claims of "infeasibility" to deny rightful data exchange even when technical solutions exist. Removing the "third party seeking modifications" condition would prevent platforms from using technical complexity as a pretext. Additional clarification regarding application of this exception would help ensure it is not interpreted in ways that unnecessarily delay secure, technically feasible data exchange, consistent with the rule's interoperability objectives.
- **Establishing "Agent-Ready" Bidirectional APIs (45 CFR § 170.315(g)(10)):** Future FHIR-based API requirements should shift from "read-only" access to supporting bidirectional workflows. This enables authorized AI agents to securely "write back" structured data, such as clinical notes or adverse events, into the system of record.
- **Modernizing HIPAA De-identification for Multimodal Data (45 CFR § 164.514(b)):** HHS should provide clearer guidance and establish pilot programs for de-identifying unstructured data types like audio, text, and ambient clinical inputs. Current frameworks have fallen behind in providing the guidance ideally required for modern AI technologies to refine models while maintaining rigorous privacy standards.

Payment Policy Prioritization

CMS should prioritize payment models that reward clinical insight over documentation volume. CMS has made sustained progress in transitioning from volume-based reimbursement toward value-based and outcomes-driven models. AI-enabled clinical infrastructure can help accelerate and operationalize these existing policy objectives.

- **Outcome-Aligned Payments (OAPs):** Shift toward recurring payments for managing qualifying conditions, with full payment tied to measurable health outcomes rather than specific billable activities.
- **Transcript-Grounded Documentation for Risk Adjustment:** Incentivizing the use of high-fidelity, auditable AI documentation can better capture patient complexity, which is often missed in traditional claims data. AI can identify and structure critical non-clinical

factors such as housing, food, or transportation insecurity that directly impact health outcomes but are rarely coded in standard documentation.

- **CMMI Pilots:** Explore Innovation Center (CMMI) demonstration authority under Section 1115A of the Social Security Act to test AI-enabled administrative simplification models. Carefully structured pilots could measure impacts on clinician time, documentation integrity, risk adjustment accuracy, and patient outcomes, building on existing Innovation Center efforts.
- **AI-Driven Benchmarking and Metrics in APMs:** Incorporate AI solutions to objectively determine if Alternative Payment Models (APMs) are effectively contributing to value-based care goals by analyzing large datasets of de-identified clinical encounter documentation. Additionally, consider incorporating ambient AI documentation metrics into Quality Payment Program (QPP) improvement activities.
- **Payment Policy details:** Clarify that AI-assisted documentation tools that improve coding accuracy and reduce administrative burden are permissible and compliant within Medicare Advantage risk adjustment and Medicare FFS documentation standards.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security) . What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Data must be clinically useful to train robust AI, but current regulatory standards for anonymizing that data are struggling to keep pace with modern technology. Abridge recommends that HHS prioritize a modernized framework for de-identifying unstructured multimodal data, such as audio, text, and ambient clinical inputs to resolve legal ambiguity and accelerate safe innovation.

HHS should move beyond categorical removal lists toward a risk-based, standards-driven approach that emphasizes auditability and security controls.

- **Create "Privacy-Preserving" Evaluation Sandboxes:** HHS should establish regulatory sandboxes where developers can test models on de-identified real-world data under strict oversight. These environments would allow for subject matter expert feedback loops to ensure models perform accurately before full production deployment.
- **Modernize Standard for Multimodal Data (45 CFR § 164.514(b)):** HHS can clarify acceptable practices for obfuscating multimodal inputs and develop federal standards

for "trustworthy AI documentation" that align technical safeguards like encryption with ethical requirements like transparency and auditability.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

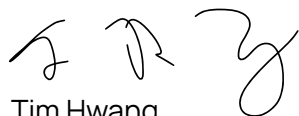
Enhanced interoperability is the primary catalyst for shifting AI from a series of "silos" to an integrated clinical partner. By breaking down barriers between disparate systems, the healthcare industry can unlock significant market opportunities, fuel critical research, and accelerate AI development. For AI adoption to transition from isolated departmental pilots to comprehensive, enterprise-wide strategies, enhanced interoperability is essential.

Standardized data exchange would allow third-party AI "agents" to act on behalf of clinicians, which opens markets for automated documentation, prior authorization, and real-time clinical decision support. A centralized, platform-based model minimizes the need for multiple direct connections, allowing hospitals to add new AI solutions without overwhelming IT staff.

Abridge thanks the HHS Office of the Deputy Secretary and ASTP/ONC for the opportunity to provide commentary on this critical RFI. We remain committed to partnering with HHS and the broader healthcare ecosystem to advance the responsible, and effective deployment of AI in clinical care, ensuring these transformative technologies deliver tangible improvements for patients and providers alike.

We are eager to collaborate further and contribute to the ongoing dialogue. For any inquiries or additional information, please reach out to Damika Barr at govaffairs@abridge.com.

Sincerely,



Tim Hwang

General Counsel, Abridge